DOCTOR-SIDE PRE-VISIT BRIEF

The 60-second delta.

The doctor has two minutes.

We give back the first sixty seconds.

A one-page pre-visit brief for Indian follow-up clinics. Every line sourced.

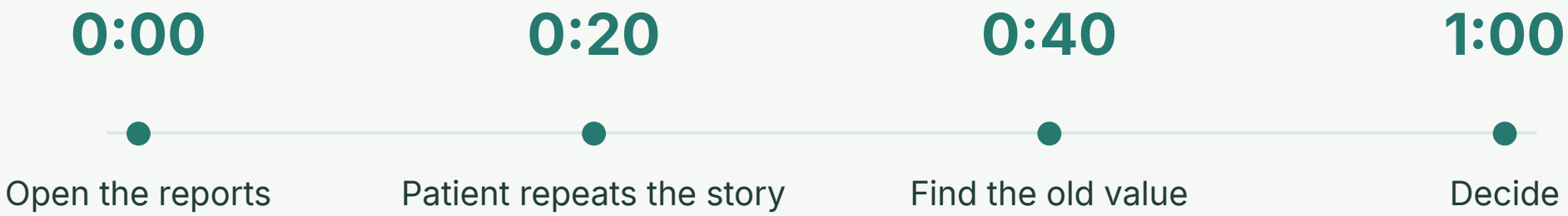


From a bag of reports to a decision.

PROBLEM

The reconstruction

Two minutes, a bag of reports, and a question the record does not answer.



The doctor should not have to reconstruct the record to see what changed.

Two minutes: nine-clinic government OPD study, IJCMH 2019.

SOLUTION

The delta

Four blocks. Three decisions. One page. The doctor decides.

01

Changed

7 items

New results, new medicines, dose changes, new events.

02

Unresolved

3 items

Results not reviewed, referrals open, questions unanswered.

03

Conflicts

2 items

The patient says one thing. The record says another.

04

What we do not know

3 items

Missing pages, partial dates, units that cannot merge.

Synthetic case. Inside a block, every line opens its original page with the line highlighted.

INNOVATION

The delta, not the record

Three things an EMR does not produce.

THE UNIT	What changed, not everything that exists.
THE SOURCE	Every line opens its original page.
THE REFUSAL	What we do not know is a block, not a gap.

A record stores. A summary narrates. A delta decides.

IMPACT

Sixty seconds back

What the doctor gets, and what we still have to measure.

THE VISIT, ALL OF IT

2 minutes

Whole consultation. Nine-clinic government OPD study, IJCMH 2019.

THE DELTA READ, INSIDE IT

42 seconds

One timed demo run on the synthetic case. It replaces the reconstruction, not the visit. Target is under 60 seconds.

Time to brief

Under 60 seconds

TARGET

Delta completeness

Share of required items

TARGET

Unresolved closed

Within 7 days

TARGET

Doctor confidence

Rated after each brief

TARGET

All four are targets, not measured results. The pilot runs a manual baseline and the delta arm on the same cases.

FEASIBILITY

Built, not mocked

Ingest to approval, in five steps that exist in the product.

01
Ingest
PDFs, photos, forwards

➡

02
Extract
Facts with evidence spans

➡

03
Bind evidence
Every fact to a source line

➡

04
Reconcile
Current against prior state

➡

05
Human approve
Accept, edit or flag

Existing backend, web and Android. Synthetic demo.

Versioned facts. Source binding on every line. Audit trail. Identity firewall. Human approval before any note is signed.

No ABDM integration claim. No WhatsApp integration claim. Documentation and retrieval aid.

BUSINESS MODEL

One workflow, one buyer

Diabetes follow-up in one Tier 2 cluster, then the rest.

BEACHHEAD

Diabetes follow-up in one Tier 2 cluster.

BUYER

Clinic owner, or hospital quality head.

PRICE

₹1,500 per doctor / month.

EXPANSION

Maternal, cancer, government OPD.

Pilot price fixed for eight weeks. At ₹1,500 per doctor per month, flat clinic pricing is about US$25 to 50 million. Value-based and government contracts are the larger path.

TEAM AND ASK

Three roles, one ask

Clinical access, a working build, a measurable pilot.

Clinical lead	Clinical plausibility, the delta rubric, the pilot sites.
Engineering lead	Pipeline, evidence binding, pilot instrumentation.
Design and product	The brief, the demo, the measurement.

THE ASK

Three clinics.

100 follow-ups.

Eight weeks.

The doctor has two minutes. **We give back the first sixty seconds.**